

إسم المريض رابعياً : بسنت توفيق عبد المحسن ابراهيم

المحافظة : المنوفيه

الحي / المدينة : اشمون

العنوان : -----

رقم البطاقة : 30208221702128 رقم التليفون : 01026267576

تاريخ تسجيل الإستمارة : 05-10-2026

العمر(تاريخ الميلاد) : 22-08-2002 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي
التوقيع: (-----)

المقرر بما فيه

التوقيع: (-----)

التشخيص الإكلينيكي : سرطان القنوات المرارية

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان القنوات المرارية

خطة العلاج : أورام كيماوي علاج كيماوي خط ثاني [4838] قسم العلاج : الاورام

التعليق على التشخيص : -----

GR 1-4
روستاق
لدى
موسى
صحة الاياد المرفقة
للجنة مدني

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

تتعمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس

وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / د ----- / د ----- / د. | ----- / د. |

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

قسم :

نَفَقَات :

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: بسنت توفيق عبد المحسن ابراهيم National ID: 30208221702128	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

A patient of bile duct cancer for Second-line chemotherapy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

٢٠٢٥/٠٤ ٣٠٢٠٨٢٢١٧٠٢١٢٨

نظام حقوق

متزوجة مسلمة أنثى

احمد طارق احمد عبدالعزيز

البطاقة سارية حتى ٢٠٢٢/٠٤/١٩

١٢



جمهورية مصر العربية

بطاقة تحقق الشخصية

بسنت

توفيق عبدالمحسن ابراهيم

صراوه

مركز اشمون - المنوفية

٢٠٠٢/٠٨ ٣٠٢٠٨٢٢١٧٠٢١٢٨

LC2113325



atient Name	بنت توفيق عبدالمحسن 0	Code Number	YK 328263
ix/Age	Female 23Y	Patient ID	24074
egistered Date	24/06/2026 10:34:42	Referred By Dr.	Out Patient
alidation Date	14/07/2026 16:05:25	Branch	Gustave Roussy
eporting Date	7/14/2026, 8:25:50 AM		

Pathology Report

Pathology Report

Clinical Data: Hepatic focal lesion in a patient with PET-CT showing metabolically active diffuse left breast lesions, metabolically active nodal (left supra-clavicular, left axillary and abdominal), hepatic and osseous deposits.

Nature of specimen: Referred block & slide (Tru-cut biopsy).

Gross Examination:

One referred block & one referred (H&E) stained slide were received labelled by (21337-26).
Technique:

- Sections prepared from paraffin block for immune-staining using Bench-Mark ULTRA system for:

CK7 (SP52), CK20 (SP33), CDX-2 (EPR2764Y), GATA3 (L50-823), SOX10 (SP267), PAX8 (MRQ-50), CD10 (SP67), ER clone (1E2), PR clone (SP1), Her2 clone (4B5), CK19 & Glypican-3.

Microscopic Examination:

Histopathological examination reveals cores of hepatic tissue showing infiltration by tumor formed of malignant acini lined by moderately pleomorphic epithelial cells with increase nucleo/cytoplasmic ratio and eosinophilic cytoplasm amid desmoplastic stroma.

** IPT (Controls showed appropriate reactivity):

- CK7: Diffusely positive in tumor cells.
- CK20: Negative in tumor cells.
- CDx2: Negative.
- GATA-3: Negative.
- SOX-10: Negative.
- PAX-8: Negative.
- CD10: Negative.
- GLYPICAN-3: Negative.
- CK19: Positive in tumor cells.
- ER: Negative nuclear staining in tumor cells.
- PR: Focal weak positive staining in 20% of tumor cells.

Name: <u>بسنت توفيق عبدالمحسن ابراهيم</u>			
MR. : <u>11606</u>			
Detailed Medical Report			
			السيد
تحتية طبية وبعد,,			
		إيماء للطلب المقدم من	
		بقسم:	
بشأن تقرير طبي للمريض		رقم طبي:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient is a 24-year-old with a negative family history who was pregnant nine months ago. A PET scan demonstrated a left supraclavicular lesion with SUV 2.1, a left breast area of diffuse metabolically active nodular densities measuring 3 cm with SUV 6.3, a left axillary lymph node 1 cm with SUV 1, and multiple hepatic focal lesions (HFL) the largest measuring 9.3 cm with SUV 14, as well as portahepatis, retrocaval, and aortocaval lymph nodes and bone deposits. Biopsy of the hepatic focal lesion confirmed adenocarcinoma, pending further pathology interpretation (IPT). The patient completed a palliative right-to-left hepatic radiation therapy (RT) and is awaiting the final pathology report (IPT). Pathology review (IPT) on CK7-positive tissue identified a moderately differentiated adenocarcinoma, with differential diagnosis including cholangiocarcinoma versus pancreatic carcinoma. A clinical pharmacy note recorded the patient's weight at 56 kg, height 162 cm, and surface area 1.58 m² for dosing of gemcitabine 1580 mg and cisplatin 55 mg; laboratory results were acceptable and obtained externally. The patient presented with multiple abscesses and vaginal candidiasis. On cycle 1 day 8 of gemcitabine and cisplatin, supportive therapy was provided, including a proton pump inhibitor, granulocyte colony-stimulating factor (G-CSF), and antimicrobial coverage. The multidisciplinary team considered a malignant unknown origin (MUO) with differential of cholangiocarcinoma versus pancreatic cancer. The patient has a history of a third-month abortion. Ultrasound in August 2026 showed a bulky uterus with hepatic focal lesions and chronic calculous cholecystitis. A trans-vaginal ultrasound was ordered. The plan was to administer gemcitabine and cisplatin on cycle 2 day 1, with continuation on cycle 2 day 8 in the inpatient setting. On cycle 2 day 8, the patient received gemcitabine 1500 mg and cisplatin 53 mg under therapeutic coverage for an abscess; weight was 53 kg, height 158 cm, and surface area 1.5 m². Laboratory monitoring was ongoing. For cycle 3 day 1, the patient is scheduled to receive gemcitabine and cisplatin, with pending laboratory results; vaginal candidiasis has resolved. The patient will also initiate pembrolizumab (Keytruda) provided by a non-governmental organization this week. The patient has completed two cycles of gemcitabine/cisplatin and is now starting cycle 3 day 1.			
تحريراً في ,			
التاريخ :		21-09-2026	
الوقت :		حنان عزت شفيق ناودروس	
الطبيب المعالج :		يعتمد ,	

Patient Name	بميت توفيق عبدالمحسن 0	Code Number	YK 328263
Sex/Age	Female 23Y	Patient ID	24074
Registered Date	24/06/2026 10:34:42	Referred By Dr.	Out Patient
Validation Date	14/07/2026 16:05:25	Branch	Gustave Roussy
Reporting Date	7/14/2026, 8:25:50 AM		

- Her2: Negative membrane staining in tumor cells.

Diagnosis:

Hepatic focal lesion, Tru-cut biopsy:

Histomorphological features of small acinar moderately differentiated adenocarcinoma, with immunohistochemical expression consistent with pancreatic-biliary origin (Intrahepatic cholangiocarcinoma versus pancreatic metastasis).

- Tightly correlate with clinical and radiologic findings, as well as metastatic work-up.

Signature

Prof. Dr. Maha Guimei.

A. Prof. Dr. Rana El-Saeed.



شقة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mier El Qadema - Cairo, Egypt

Pathology Unit

21337

السن : 24

تاريخ الحضور : 08-06-2026

تاريخ الاستلام : 13-06-2026

فرع : B-El Bagour

إسم المريض : بسنت شوقي عب المحسن

محول من أ.د : الشيماء محمود الحنفي

رقم الزيارة : 69826504068

NATURE OF SPECIMEN:

Guided core biopsy from liver lesion.

GROSS:

Two cores of collective length 2.7 cm, long. Totally embedded.

MICROSCOPY:

Core biopsy shows hepatic tissue infiltrated by malignant tumor formed of irregular glandular formations separated by a fibrous stroma. Glands are lined by malignant epithelial cells have large vesicular nuclei with pale eosinophilic cytoplasm. There is moderate anaplasia and mitosis. Metastatic work-up to exclude metastasis from elsewhere (especially breast, GIT, female genital and lungs) is recommended before considering the possibility of a primary hepatic origin. Marker study (CK7, CK20, GATA-3, PAX-8, TTF-1, CDX-2, HepPar1 and synaptophysin) may be of help in determining the primary origin.

DIAGNOSIS:

Hepatic lesion, guided core biopsy: Adenocarcinoma, grade 2. Recommended for metastatic work-up and marker study to identify the primary organ of origin.

Akram
Nouh

Prof. Dr. M. Akram Nouh

Habiba

Dr. Habiba Elfendy

M El Shenawy

Dr. Mohamed El Shenawy

Authentication Date: 13-06-2026

Name : Basent Tawfik Abd Al-Mohsen	Pat. ID: 301018
Date : 08/06/2026	Ref. Doctor: الشيماء محمود الحنفى

Dear sir :

2 true cuts from liver lesion for histopathological evaluation and
immunophenotyping

Metastatic process.

Dr hatem Moharram

Imaging report

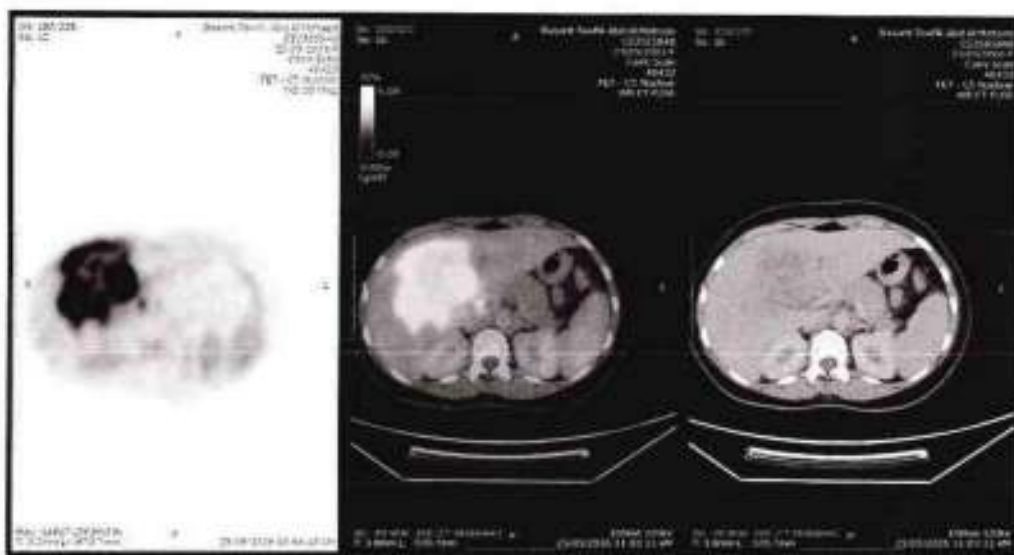


كايرو سكان
للمسحوق والتصوير
CAIRO SCAN
Radiology & Labs

Pat. Name: Basant Tawfik Abd Al Mohsen

Pat. ID: 3565848

Date: 25/05/2026



MUSCULO-SKELETAL

- Metabolically active randomly distributed osseous lesions are seen involving skull base, multiple dorso-lumbar vertebrae, scattered ribs bilaterally, multiple pelvic bones including the sacrum, sternum, both scapulae and both femora with SUVmax11.7 recorded at the skull base lesion



OPINION



Pat. Name: Basant Tawfik Abd Al Mohsen

Pat. ID: 3565848

Date: 25/05/2026



CHEST:

- Dense left breast parenchyma showing metabolically active diffuse nodular densities involving whole its glandular structure reaching about 3 x 2.9 cm in max axial diameter with SUVmax6.3
- Low grade metabolically active left axillary lymph nodes. The largest measuring about 1 cm in max axial dimension with SUVmax1
- No metabolically active right breast soft tissue lesions . No metabolically active right axillary & pectoral lymph nodes Subcentimetric right axillary lymph nodes with no pathologic FDG uptake (non specific).
- No metabolically active sizable pulmonary nodules, masses, infiltrations or bronchiectatic changes and the detected tiny few pulmonary nodules not exceeding 0.3 cm show no significant FDG uptake for follow up .
- No pleural or pericardial sac collection.
- No metabolically active grossly enlarged hilar or mediastinal lymph nodes.
- Normal tracer distribution at the myocardium

Imaging report



كايرو سكان
CAIRO SCAN
Radiology & IAS

Pat. Name: Basant Tawfik Abd Al Mohsen

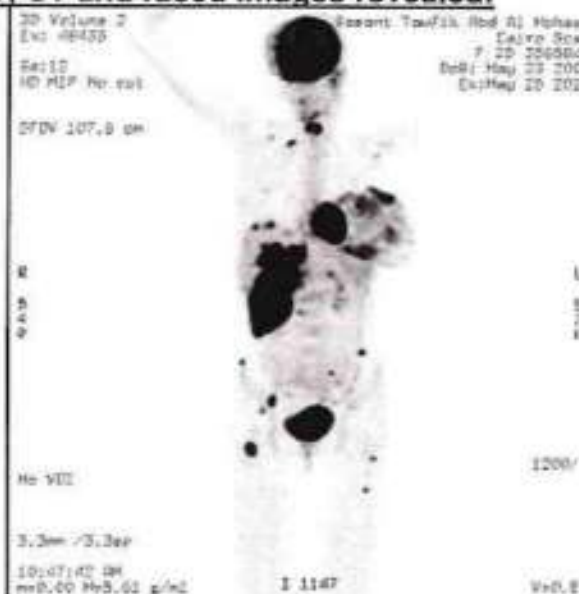
Pat. ID: 3565848

Date: 25/05/2026

¹⁸F FDG Whole Body PET/CT

- Whole body ¹⁸F-FDG images from skull to the mid thighs with the arms extending above the head were obtained
- CT was acquired for the purpose of anatomic localization and attenuation correction.
- IV contrast was administered.
- Data was processed and displayed according to departmental protocol.

Review of PET, CT and fused images revealed:



HEAD AND NECK

- metabolically active left supra-clavicular lymph node measuring about 1.2 cm in max axial dimension with SUVmax 2.1
- Patent nasopharyngeal and oro-pharyngeal air column.
- Normal appearance of the parotid and submandibular salivary glands detected
- No metabolically active thyroid focal lesions detected
- No hypermetabolic gross brain lesions detected.

Imaging report



كايرو سكين
للأشعة والتصايل
CAIRO SCAN
Radiology & LABS

Pat. Name: Basant Tawfik Abd Al Mohsen

Pat. ID: 3565848

Date: 25/05/2026



ABDOMEN AND PELVIS:

- Metabolically active multiple variable sized hepatic focal lesions are seen scattered at both lobes, the largest measures about 9.3 x 8.1 cm in max axial diameter with SUVmax14.2
- Metabolically active portahepatis, para-aortic, retrocaval, portocaval and aorto-caval lymph nodes. The largest measuring about 1.5 cm in max axial dimension with SUVmax4.9
- No hypermetabolic pancreatic, splenic, suprarenal or renal focal lesions detected.
- No metabolically active grossly enlarged pelvic lymph nodes.
- Aorta and IVC were unremarkable
- No ascites or pelvic collection.
- Normal tracer distribution within the gastrointestinal and genitourinary systems without focal areas of abnormal metabolism.
- Bilateral adnexal cysts with no pathological tracer uptake The largest measuring about 3.5 cm in max axial dimension



كايرو سكين
مؤسسة البحث
CAIRO SCAN
Radiology & IASIS

Imaging report

Pat. Name: Basant Tawfik Abd Al Mohsen

Pat. ID: 3565848

Date: 25/05/2026

- Metabolically active diffuse left breast lesions for correlation with tumor markers and histopathology & immunophenotyping
- metabolically active nodal (left supra-clavicular , left axillary and abdominal) , hepatic and osseous deposits
- No gross hypermetabolic lesions detected at the rest of the scanned body.

Best Regards,
Prof. Dr. Maha Abd Elkareem
Professor of Nuclear Medicine
Kasr Al Ainy, Cairo University

*For any inquiries regarding the findings in the report please contact us on
radreport@cairoscan.com.eg*

الطبيب المعالج: أ.د/ وليد غريبه
العمر: 24 عام

الإسم: بسنت توفيق عبدالفتاح
2026.05.29

Both Breast U/S

Breast sonographic were examined by high resolution 7.5 – 10 MHz U.S. probe in supine & sitting position in longitudinal, axial & radial planes along with axillary scanning:

- Both retroareolar areas showed dilated ducts reaching the retroareolar area about **4.5 mm** and partially showed moving echoes, no evidence of intraductal gross masses ... **ductectasia (normal with lactation).**
- **Normal** skin thickness.
- **Normal** echogenicity of the breast fat including subcutaneous, parenchymal, reteroparenchymal fat.
- **Normal** orientation of the Cooper's alignments without distortion.
- **Normal** parenchymal echogensity **without masses, cysts or calcifications.**
- Normal pectoralis major muscle.
- **The left axilla showed few non-significant enlarged lymph nodes**
- **Normal** axillary arteries and veins in both sides.

Conclusion

Unremarkable study
(BIRAD- I classification)

Much Obliged, Yours
Dr. Osama E. Alakb.M.sc.

Name: Basant Tawfik Abd Al Mohsen	Date: 27 Sep 2026
ID: 3565848	Referring:

Whole body F-18 FDG PET/CT EXAMINATION:

Clinical History:

- 25 years old patient with hepatic adenocarcinoma
- PET/CT requested for assessment.

Technique:

- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed simultaneously and used for attenuation correction, anatomical localization and diagnosis.

Report

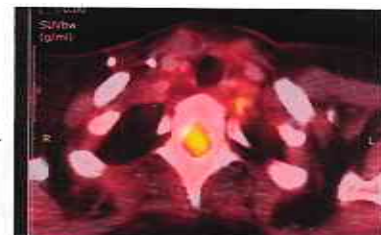
Compared to the previous PET-CT dated: 25-05-2026

Head and Neck:

- * Stationary course of the old metabolically active left supra-clavicular lymph node still measuring about 1.2 cm with SUVmax 2.3(compared to



SUVmax~2)



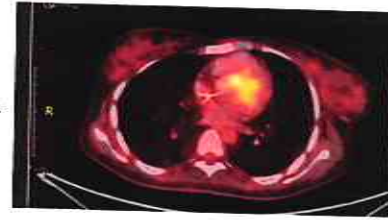
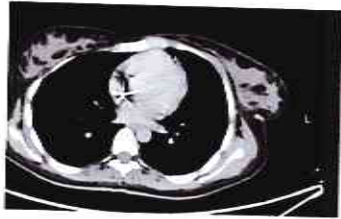
- * Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.
- * The brain shows uniform tracer distribution with NO hypermetabolic focal lesions.

Chest:

- * Still noted Dense left breast parenchyma showing low grade metabolically active diffuse nodular densities of SUVmax1.2 (was with SUVmax~6.3)

Name: Basant Tawfik Abd Al Mohsen
ID: 3565848

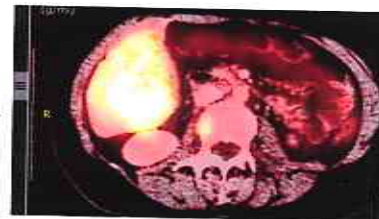
Date: 27 Sep 2026
Referring:



- * Currently No hypermetabolic axillary or mediastinal lymphadenopathy.
- * No FDG avid lung nodules or masses still noted tiny few pulmonary nodules not exceeding 0.3 cm showing no significant FDG uptake for follow up .
- * There are no active pleural abnormalities.
- * Physiologic FDG uptake is noted in the heart.

Abdomen and Pelvis:

- * Mild metabolic regression of the old Metabolically active multiple variable sized hepatic focal lesions seen scattered at both lobes, the largest measures about 8.3x6.5 cm with SUVmax~11 (was 9.3 x 8.1 cm with



SUVmax14.2).

- * Regression of the old Metabolically active porta hepatis, para-aortic, retrocaval, portocaval and aorto-caval lymph nodes. The largest measuring 1.3 cm with SUVmax~2.1 (was 1.5 cm with SUVmax4.9)

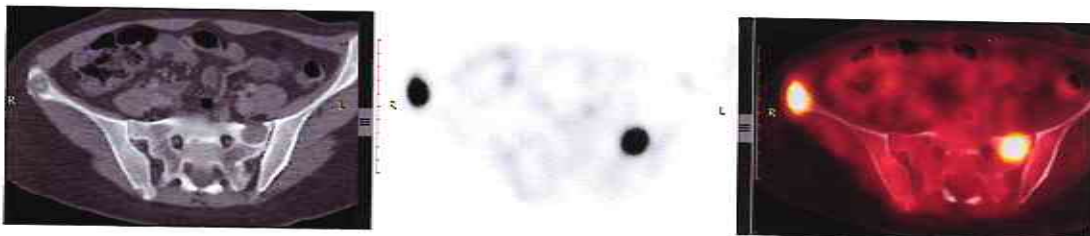
Name: Basant Tawfik Abd Al Mohsen	Date: 27 Sep 2026
ID: 3565848	Referring:



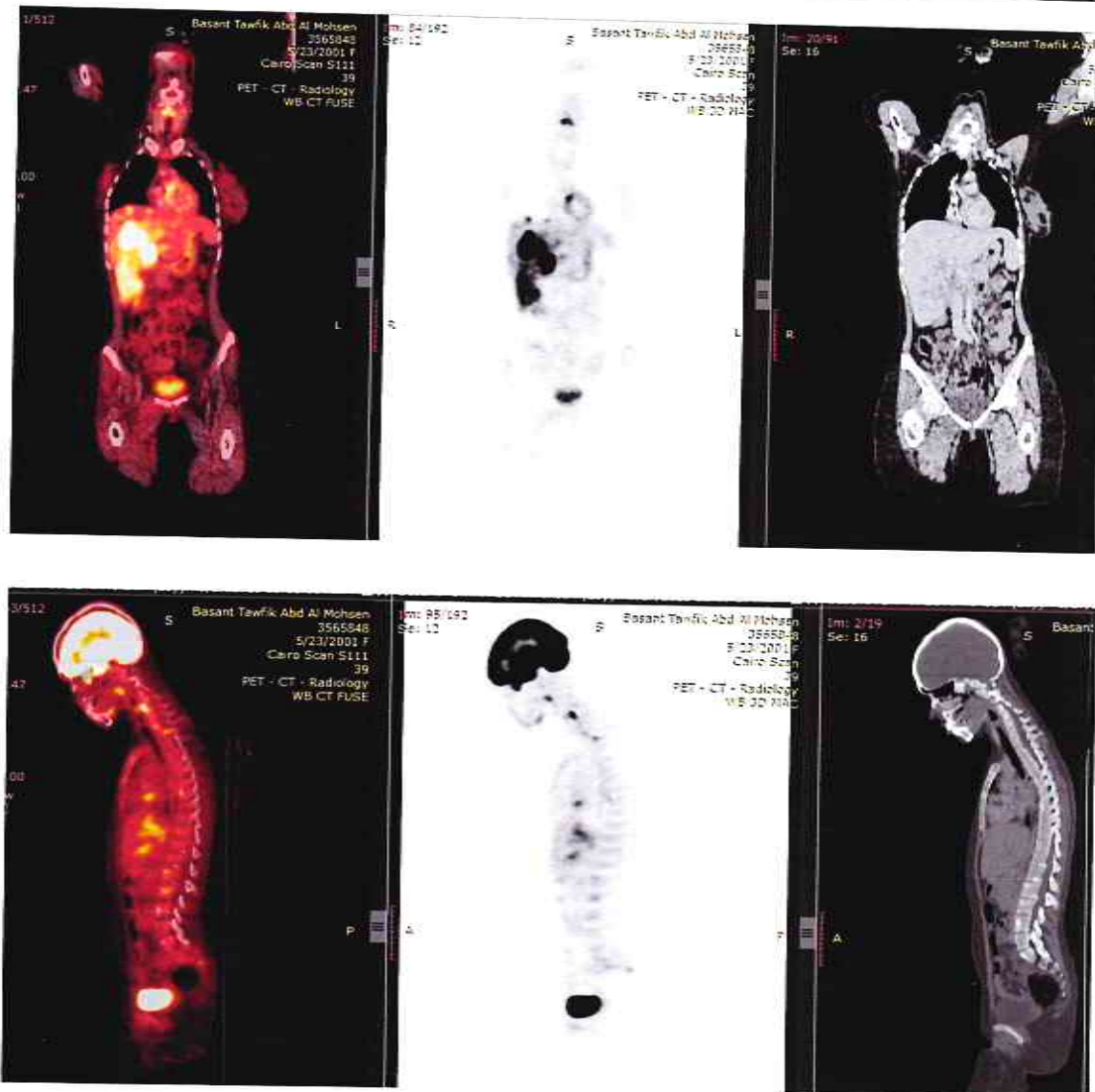
- * Physiologic diffuse FDG uptake is seen over average size spleen with homogenous parenchymal texture and no FDG avid focal lesions seen.
- * The pancreas shows normal FDG avidity.
- * There are no active adrenal nodules.
- * Physiologic FDG excretion is seen in the kidneys and bladder.

Musculoskeletal:

Mild metabolic regression of the old active osseous lesions involving skull base, multiple dorso-lumbar vertebrae , scattered ribs bilaterally, multiple pelvic bones including the sacrum, sternum, both scapulae and both femora with SUVmax10 at right iliac bone(compared to SUVmax~11.7)



Name: Basant Tawfik Abd Al Mohsen	Date: 27 Sep 2026
ID: 3565848	Referring:



Conclusion:

Follow up PET-CT study showing:

- ***Mild regression of the old Metabolically active multiple variable sized hepatic neoplastic focal lesions.***

Name: Basant Tawfik Abd Al Mohsen	Date: 27 Sep 2026
ID: 3565848	Referring:

- ***Regression of the old Metabolically active abdominal lymph nodes.***
- ***Mild metabolic regression of the old metastatic osseous lesions.***
- ***stationary course of the old mildly active left supra-clavicular lymph node.***
- ***No newly developed FDG avid metastatic lesions detected.***

Thank you for referral

***Dina Thabet , MSc
Nuclear medicine specialist
National Cancer Institute.***

***Ahmed Zaher , MD
Professor of nuclear medicine
National Cancer Institute***